



KPV RESEARCH HUB · RESEARCH USE ONLY

CALM THE GUT. CLEAR THE SKIN.

FOR MEN & WOMEN. Calm gut inflammation, heal skin, and cool down your immune system.

[Get KPV vial at BioLinx](#)

[Read the guide first](#)

Also shown to help with: [ibs](#) · [crohn's disease](#) · [eczema](#) · [psoriasis](#) · [acne](#)
Once daily · time of day doesn't matter · Difficulty: beginner

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RESEARCH SHOWED THAT...

Gut

↓ inflammation
blocks the exact pathway
behind intestinal flare-ups.

Gastroenterology, 2007

KPV turns off the inflammation
signals — the chemicals that
cause gut pain and swelling —
in human cell studies.

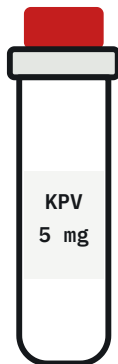
PMC, 2008



STEP 1 OF 4 · WHAT TO BUY

SHOPPING LIST

Easy-to-follow shopping list to safely practice KPV. Match your commitment level — don't skip anything.



VIAL 1 · PEPTIDE

PEPTIDE · ONE ORDER

KPV VIAL

Calm gut inflammation, heal skin, and cool down your immune system. Common starting dose: **0.25 mg**, once daily, time of day doesn't matter.

[Get KPV vial at BioLinx](#)

biolinxlabs.com/best-sellers

HOW MANY VIALS TO BUY · BEST VALUE BY GOAL

GOAL	VIAL	QTY	WHERE
1 MONTH	5 mg vial	2×	<u>Best sellers</u> not listed yet · biolinxlabs.com/best-sellers
FULL CYCLE RECOMMENDED	5 mg vial	5×	<u>Best sellers</u> not listed yet · biolinxlabs.com/best-sellers

MATERIALS ≈ \$41 ON TOP — FULL LIST ON THE NEXT PAGE



PRODUCTS ARRIVED?

QUICK SUMMARY OF YOUR LAB ITEMS:

TWO VIALS IN THE BOX • KPV VIAL + BACTERIOSTATIC WATER 10 ML

VIAL 1 **KPV vial.** Arrives as a powder. Don't touch it until you've read page 4.

VIAL 2 **Bacteriostatic water — 10 mL.** This is what turns the powder into a liquid.
Sterility and pH matter — don't substitute.

GROCERY LIST • ≈ \$41

ITEM	SPEC	QTY	WHERE
INSULIN SYRINGES BUY	1cc U100, 30G, 5/16"	50–100 ct	Any pharmacy
ALCOHOL PREP PADS BUY	Any brand, 70% isopropyl	50–100 ct	Any pharmacy
SHARPS CONTAINER BUY	1 quart, for needle disposal	1×	Any pharmacy
BAC WATER BUY	Bacteriostatic water, 10 mL — sterility and pH matter, don't substitute	1 vial	<u>See it</u> biolinxlabs.com/products/bacteriostatic-water-10-ml

Any brand works for syringes, pads and sharps — but not BAC water. Use 2 fresh syringes and 2 fresh pads every session: one to mix, one to inject.

HOW TO STORE IT

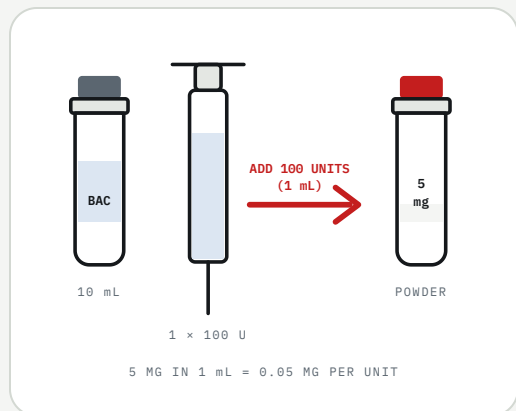
POWDER	Freezer — lasts years
RECONSTITUTED	Fridge — about 28 days. Write the mix date on the vial.
WATER	Room temperature — check the expiration date



STEP 2 OF 4 · RECONSTITUTION – FROM POWDER TO LIQUID

LET'S “FORM” YOUR PEPTIDE!

“Form” means “reconstitute”. Your product arrives as a powder — you need to liquify it before “you begin your research.” 100 units = one full syringe. **5 mg vial** → **add 100 units (1 mL)**. Every dose in this guide assumes exactly that ratio, so measure it — don't eyeball it.



YOUR SIMPLIFIED UNITS

DOSE	FILL TO
0.25 mg	5 units
0.5 mg	10 units
0.75 mg	15 units
1 mg	20 units

Change the vial, water or dose →
[interactive calculator](#)

1

Remove the plastic caps & wipe both rubber vial tops

Fresh alcohol pad on the KPV vial and the bac water vial.

2

Draw 100 units of bac water

One full pull on a 100-unit insulin syringe (1 mL total).

3

Inject it slowly down the side of the 5 mg vial

Let it run down the glass. Don't blast the powder.

4

Swirl gently. Never shake. Place it in the fridge!

Slow and gentle wins this one — shaking wastes peptide. Discard that syringe into the sharps container. Let it rest until completely clear (36–46 °F); use within ~28 days; never freeze it.



STEP 3 OF 4 · INJECT

WHERE TO INJECT

Time to inject · clean, pinch, poke, done. Once daily, time of day doesn't matter.

1

Take KPV out of the refrigerator and extract your dose

Your first dose is **0.25 mg = 5 units** on a 100-unit syringe. Wipe the vial top with a fresh alcohol pad, let it air-dry a few seconds, then draw your units.

2

Pinch a fold of skin on the stomach + use an alcohol wipe

Two inches away from the belly button. Wipe with a fresh alcohol pad, let it air-dry, and don't touch that spot again before injecting. Rotate sites.

3

Needle in at the base of the fold

Push the plunger slowly and steadily. Hold for a few seconds, then withdraw.

4

That's it — you just did your first shot!

Drop the needle straight into the sharps bin. Vial goes back in the fridge.



STOMACH · PINCH A FOLD · NEEDLE AT THE BASE

WATCH FIRST

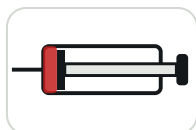
Mixing and injecting from start to finish, on video: professorpeptides.com. Use 2 fresh syringes and 2 fresh pads every session — one to mix, one to inject.



STEP 4 OF 4 · DOSING & TIMING SCHEDULE

KPV SCHEDULE (PROTOCOL)

“Schedule” means “protocol”. Everything below assumes the 5 mg vial was reconstituted with exactly 100 units (1 mL), read off a standard 100-unit insulin syringe.



FIRST DOSE

How much, how often & when?

WHEN

Time of day doesn't matter. Once daily.

HOW MUCH

0.25 mg to start.

Dose runs in on/off blocks with breaks between — this is **cycling**, not titration. Full schedule on page 9.

UNITS

Extract **5 units** on a 100-unit insulin syringe.

5 units = 0.25 mg. Exact variables: 100-unit syringe · 5 mg vial reconstituted with 100 units · 0.05 mg per unit.

HOW OFTEN

Once daily. Hold each step for its full block before moving up.

CYCLE

On-cycle 4–8 weeks · off-cycle 2–8 weeks.

Don't rush it — start at the lowest amount, hold each step for its full block, and only step up if the current dose is comfortable. Most people never need the top doses.



STEP 4 OF 4 · DOCUMENT

WHAT TO EXPECT + SIDE EFFECTS

Mostly mild, mostly early, mostly right after a dose increase. Know the three tiers before the first shot, not after.

NORMAL / EXPECTED

First few days

- Mild redness or irritation at the injection site
- Transient flushing or warmth after the shot
- Slight fatigue or a mild headache in the first days

COMMON, WORTH NOTING

Manage it, don't push the dose

- Mild nausea — most often when the daily dose goes above the starting range
- Brief dizziness or light-headedness early on (blood-pressure related)
- Hot or cold flushes that pass within the hour

STOP & CALL A DOCTOR

Don't push through it

- Dizziness that doesn't pass, or fainting
- Persistent nausea or vomiting
- Signs of an allergic reaction — swelling, rash, difficulty breathing
- Injection site that becomes hot, hard, very painful or oozes (possible infection)

NOTE: for this one it is **HIGHLY RECOMMENDED** to start at the lowest amount first and work up over time, as needed. KPV has no human safety trials — its profile is extrapolated from alpha-MSH. Keep doses low, stop at anything more than mild, and talk to a doctor first if you are pregnant, breastfeeding, or have a liver condition.



FIRST-TIME TL;DR

RECAP: CHECKLIST

Everything from pages 2–7, in the order it happens.

BEFORE

- ☐ Vial reconstituted with exactly 100 units of bac water, resting in the fridge, mix date written on it
- ☐ Supplies laid out: new syringe, alcohol pad, sharps container
- ☐ Dose set at **0.25 mg = 5 units** — not higher, no matter what a forum says

DURING

- ☐ Inject once daily, time of day doesn't matter — stomach, rotate the spot
- ☐ Note the date and dose somewhere — a phone note is fine

AFTER

- ☐ Expect possible redness, mild irritation or a headache — it settles
- ☐ Mild nausea can happen after a dose increase — sit tight, it typically passes
- ☐ If anything feels more than mild — dizziness, real nausea, allergic reaction — stop and get medical advice

NEXT BLOCK

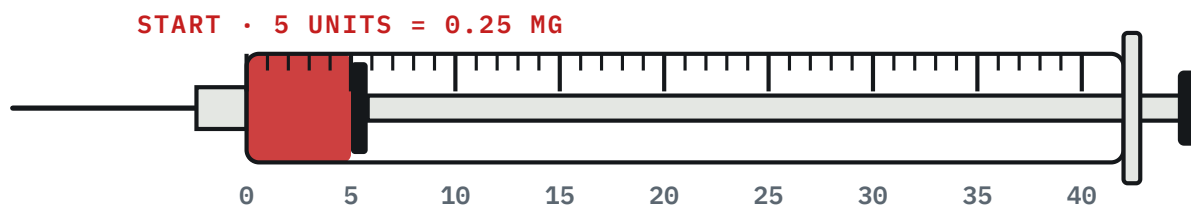
- ☐ Hold each step for its full block before stepping up — never skip a step (on-cycle 4–8 weeks · off-cycle 2–8 weeks)
 - ☐ Only step up if the current dose is tolerated; if side effects are rough, stay another block
 - ☐ Fresh syringe, fresh pad, needle in the sharps bin — every single time
-

UNIT CHEAT-SHEET · 100-UNIT SYRINGE · 5 MG VIAL RECONSTITUTED WITH 100 UNITS

HOW TO CYCLE (RAMP-UP & CYCLE SCHEDULE)

Assumes the 5 mg vial was mixed with exactly 100 units, so **1 unit = 0.05 mg**.

Units shown are per shot.



WHEN	DOSE	SYRINGE · PER SHOT	NOTES
Month 1	0.25 mg per day	5 units	START HERE
Months 2–3	Off	—	REST
Month 4	0.375 mg per day	7.5 units	
Months 5–6	Off	—	REST
Month 7	0.5 mg per day	10 units	MAX DOSE
Months 8–9	Off	—	REST

Rule of thumb: don't move up a step until you know how the current dose feels. Only step up if the previous block was tolerated. Most people never need to go past the middle of the ramp.



THE KPV SOURCING GUIDE

PROFESSOR PEPS: SOURCING GUIDE (APPENDIX)

How to verify any supplier in 90 seconds. A legitimate source clears all three checks below in under two minutes — no back-and-forth required.



1

Ask for the COA

A batch-specific Certificate of Analysis — not a generic PDF reused across batches — showing purity $\geq 98\%$ via HPLC/MS.

2

Match the lot number on the vial

It must match the lot number on the COA. If they don't match, walk away.

3

Check the track record

Searchable reviews or forum mentions older than roughly six months. Brand-new storefronts with no history are the highest-risk category.

DON'T WORRY — WE'VE ALREADY DONE THE VETTING

BioLinx Labs clears the list: third-party HPLC testing, a 99%+ purity guarantee and a COA included with every order. biolinxlabs.com/best-sellers

biolinxlabs.com/best-sellers

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**STAY CURIOUS,
PROFESSOR PEPTIDES**

KPV Research Hub · companion to the email series